



CE HALO

CRNA CONTINUING EDUCATION

The First Days in the OR

A Clinical Onboarding Module

For New SRNA Students & Anesthesia Residents
Preparing for clinical practice in the operating room

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Contents

A practical clinical-onboarding guide for new SRNAs and anesthesia residents. Use it with your preceptor, local policies, and the instructions for the equipment in your room.

How to use this edition: Sections 1–3 prepare you for clinical; Sections 4–8 cover room, medication, and emergency readiness. Sections 9–10 cover the interview and anesthetic plan. Section 11 follows the first case; Section 12 covers emergence through PACU. Section 13 is the room-setup pocket card.

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1

Before Your First Clinical Day

Preparation the night before lowers your anxiety more than anything else. Preceptors expect nervousness — what they don't forgive is showing up unprepared.

The night before

- ◆ Look up your cases and patients. Review the surgical schedule, the procedure, expected position, and anticipated blood loss. Look up anything you don't recognize.
- ◆ Build your care plan. Many programs and preceptors will ask to physically see a written anesthetic care plan for each patient — have it ready in the format your school requires ([Tips for SRNAs Starting Clinicals, NARs](#)).
- ◆ Review the patient's history: allergies, NPO status, airway exam (Mallampati, thyromental distance, mouth opening, neck mobility, dentition), comorbidities, current meds, labs, and prior anesthetic records.
- ◆ Pre-formulate a plan for induction, maintenance, airway management, lines/monitors, and emergence — including a backup airway plan.
- ◆ Confirm logistics: report time, where to park, where to change into scrubs, badge/access, who your preceptor is, and where to meet.
- ◆ Sleep and eat. You may not get a break for hours.

What to bring

- ◆ Stethoscope, trauma shears, pen light, tape, and a black pen (or two).
- ◆ A pocket reference of emergency drug doses and your school's care-plan template.
- ◆ Watch with a second hand. Snacks/water for when you finally get a break.
- ◆ A small notebook to write down where things are kept in this room and this facility.

What to wear / professionalism

- ◆ Facility scrubs, a bouffant/surgical cap, clean shoes you can stand in all day, and your ID badge visible.
- ◆ Arrive early. “On time” in anesthesia means early enough to have the room fully set up and checked before the patient arrives.



2

The Morning Routine

A reliable morning starts with the people in your room, then moves through equipment, supplies, medications, and the plan for your patient.

- 1. Arrive early and meet your preceptor.** Allow enough time to prepare without rushing. Confirm your assignment, the first case, and any overnight or last-minute changes.
- 2. Introduce yourself to the OR team.** Introduce yourself to the circulating RN, scrub person or surgical technologist, and other staff in the room. Give your name and your role as an SRNA or anesthesia resident.
- 3. Make sure the circulating RN has your name.** The circulating RN needs your name and role for the intraoperative documentation/record. Offer the spelling of your name rather than assuming it is already known. Ask the RN's name, too.
- 4. Confirm the machine checkout and personally recheck.** Depending on the facility, an anesthesia technician may perform the initial checkout. I still personally recheck the machine and verify readiness because the anesthesia provider remains responsible for the equipment used. As a learner, do this with your preceptor.¹
- 5. Set up for this patient and this procedure.** Use SOAP-ME, the practical walkthrough in Section 4, and the checklist in Section 5. Think ahead about patient size, surgical location, position, warming, IV access, and postoperative oxygen.
- 6. Prepare medications and confirm emergency stock.** Prepare and label the medications needed for the case. Confirm that emergency medications are stocked and accessible; “available” does not mean that every medication must be drawn up. Follow Section 7 and your preceptor's plan.
- 7. Review the patient and coordinate the plan.** Review the chart, allergies, NPO status, consent, and any changes; use the patient-interview guide in Section 9. Discuss the plan with your preceptor and clarify positioning, anticipated blood loss, and equipment needs with the surgical team.
- 8. Know how to get help.** Locate emergency equipment and confirm how to call for assistance. Participate in the applicable patient sign-in and surgical time-out at the appropriate stages; the morning room check does not replace them.

1. APSF, provider responsibility: <https://www.apsf.org/article/new-guidelines-available-for-pre-anesthesia-checkout/>



3

The Professional Mindset: How to Show Up

- ◆ It is OK to be nervous — preceptors know. What matters is preparation and a willingness to learn ([Tips for SRNAs, NARs](#)).
- ◆ Ask for help early and often. Every machine and facility is different; asking “where is X kept?” or “can you show me this machine?” is expected, not a weakness.
- ◆ Closed-loop communication. Repeat back orders and drug doses. Confirm before you give anything.
- ◆ Verify every medication and its label before giving it, including prefilled syringes. Never give an unlabeled drug.
- ◆ Stay at the head of the bed. Your patient's airway and vitals are your responsibility every second.
- ◆ Own your mistakes immediately. Speak up about near-misses. Patient safety always outranks ego.
- ◆ Anticipate. Think one step ahead — what does the patient, surgeon, or preceptor need next?

4

Setting Up a Basic OR (SOAP-ME)

The classic memory aid for room setup is S-O-A-P-M-E. Set up and confirm each category before every case ([Pre-Anesthetic Set-Up, ProCRNA](#)).

Category		What to have ready
S	Suction	Suction on, tubing connected, Yankauer attached, tested and within reach at the head of the bed. A mandatory ASA checkout item (ASA Checkout).
O	Oxygen	Pipeline and backup O2 cylinder checked against workstation/local pressure requirements; functioning self-inflating ventilation bag at the head of the bed.
A	Airway	Correctly sized ETTs (+ one size above/below), laryngoscope handle + blades (check the light), stylet, cuff syringe, oral/nasal airways, tongue depressor, LMAs, tape, backup device. Confirm the GlideScope location.
P	Pharmacy	Case-specific medications prepared and labeled; emergency medications accessible. Not every stocked drug needs to be drawn up (Section 7).
M	Monitors	ASA standard monitors: pulse oximeter, ECG (EKG stickers), NIBP cuff (correct size), capnography (EtCO2), temperature probe, agent analysis. Set/confirm alarm limits.
E	Equipment	Machine checked, IV setup and fluids, warming devices, positioning aids, and case-specific equipment (A-line setup, fluid warmer, pressure bags).

Essential “never skip” items on every case (Missed Steps in the Preanesthetic Set-Up, Anesthesia & Analgesia 2011):
Manual ventilation device · Full machine checkout · Adequate suction · Emergency airway devices (ETT, LMA) ·
Emergency drugs · A working IV · ASA monitors.



4

How I Set Up My Room

As a CRNA with over 16 years experience, this is how I set up my room

This sequence is adapted to the patient, procedure, available equipment, and local policy. SRNAs should complete and verify the setup with their preceptor.

Start with the machine and suction

- **Machine checkout:** In many locations, the anesthesia tech performs the initial machine check. I still recheck the machine myself. I do not assume that a completed automated test or someone else's checkout replaces my own pre-use verification.¹
- **Vaporizers:** I check agent levels and fill the vaporizers as needed, using the correct agent and the manufacturer's filling procedure. I verify that filler ports are closed and complete the required checks after any change.¹
- **Suction:** I turn it on, confirm that the tubing and suction tip are connected, and test that suction is working. I keep it within reach at the head of the bed.¹

Build the airway setup

- **Intubation supplies:** I have the appropriate laryngoscope blade and handle ready, with a working light, the correct-size endotracheal tube (ETT), a cuff syringe, and a stylet.
- **Cuff and securement:** I always test the ETT cuff before use, following the device instructions, and leave it ready for insertion. I have clear medical tape available to secure the tube.
- **Airway adjuncts and recovery oxygen:** I have an oral airway and tongue depressor available, plus a nasal cannula or face mask ready for postoperative oxygen as planned. I also confirm the backup airway and manual ventilation equipment.

Have the monitors ready before arrival

- **ECG/EKG:** I attach the electrodes to the lead wires and place the prepared leads on the bed so they are ready to apply. I keep the adhesive backings on until placement.
- **Pulse oximetry and blood pressure:** I make sure I have a pulse oximeter probe and a correctly sized blood pressure cuff, with the required hose/cable and connectors. At some locations, patients arrive with a disposable cuff; I still confirm that it is compatible and can connect to the monitor.

1. APSF, pre-anesthesia checkout: <https://www.apsf.org/article/new-guidelines-available-for-pre-anesthesia-checkout/>



4

How I Set Up My Room: Continued

Plan warming, temperature monitoring, and position

- **Gloves:** I keep gloves in my size available where I can reach them.
- **Forced-air warming:** I select an upper- or lower-body Bair Hugger blanket according to the surgical site and position. I most often use the upper-body blanket, but I first ask: where on the body is the surgery taking place, and which area needs to stay accessible?
- **Temperature:** I have an esophageal or skin temperature probe available, as appropriate to the patient, anesthetic plan, and monitoring needs. Probe selection is confirmed with the preceptor rather than treating the two sites as interchangeable.
- **Prone cases:** I have the planned face pillow or prone-view head support ready, along with a flexible breathing-circuit/tube extender. I confirm the positioning and airway-equipment plan with the preceptor and OR team before the patient arrives.

Prepare IV equipment and additional lines

- **Access and fluids:** I set up the IV supplies, fluids, tubing, extension sets, and any additional lines or monitoring equipment required for the case. I check the plan for arterial access, pressure bags, and fluid warming when indicated.

Then prepare the medications

- **Case medications:** Once the room equipment is ready, I gather syringes and prepare the medications I expect to need for the case. I label each prepared syringe with the drug name and concentration, plus preparation details required by policy.²
- **Emergency and hemodynamic stock:** I confirm that my medication cart is stocked with glycopyrrolate, atropine, ephedrine, phenylephrine, epinephrine, labetalol, esmolol, and other medications required by the facility and case. I check concentrations, expiration dates, and ready access.
- **My narcotic-handling routine:** Where facility policy permits, I keep the controlled substances assigned to my case in my personal custody and usually do not draw up narcotics until they are needed. A prepared syringe carried in a pocket can accidentally leak or spill. This is my workflow preference, not a recommendation to carry loose syringes in a pocket.

For learners: Follow your facility's approved controlled-substance storage, custody, administration, reconciliation, and witnessed-waste procedures. Confirm the timing of medication preparation with your preceptor so that necessary drugs are ready without delaying care.

2. ASA, syringe labeling:

<https://www.asahq.org/standards-and-practice-parameters/statement-on-labeling-of-pharmaceuticals-for-use-in-anesthesiology>



5

Equipment & Supply Checklist

Use this with your preceptor and the case plan. Select the supplies needed for this patient; this list does not replace the machine-specific checkout.

Airway and oxygen

- ☐ Correct-size ETT, with appropriate alternative sizes available.
- ☐ ETT cuff tested; cuff syringe and stylet ready.
- ☐ Laryngoscope handle and appropriate blade(s); light/function checked; backup available.
- ☐ Clear medical tape or approved tube-securement device.
- ☐ Oral airway and tongue depressor; other airway adjuncts as indicated.
- ☐ Supraglottic airway/LMA and backup airway equipment.
- ☐ Appropriate face mask and functioning self-inflating manual ventilation bag.
- ☐ Nasal cannula or face mask ready for planned postoperative oxygen.
- ☐ Working suction with tubing and Yankauer/suction tip connected and within reach.

Monitoring

- ☐ ECG/EKG electrodes attached to lead wires and placed on the bed; adhesive backings remain on until use.
- ☐ Pulse oximeter probe and compatible cable.
- ☐ Correct-size NIBP cuff, hose/cable, and connectors. Confirm compatibility if the patient arrives with a disposable cuff.
- ☐ Temperature probe appropriate to the monitoring plan: esophageal or skin, as selected with the preceptor.
- ☐ Capnography sampling line, gas analysis, and required monitor connections.
- ☐ Neuromuscular monitoring equipment if neuromuscular blockade is planned.
- ☐ Required monitor functions and alarm settings verified.



5

Equipment & Supplies: Continued

IV access, fluids, and additional lines

- ☐ IV start supplies: catheters, tourniquet, skin prep, tape, and dressings.
- ☐ IV fluids, tubing, extension sets, stopcocks, and an IV pole.
- ☐ Additional IV access and case-specific lines as planned.
- ☐ Arterial-line setup, transducer, tubing, and flush solution if indicated.
- ☐ Pressure bags, fluid warmer, or rapid-infusion equipment when indicated.

Warming, positioning, and protection

- ☐ Bair Hugger/forced-air warming unit and a blanket selected for the surgical site and position: upper or lower body as appropriate.
- ☐ Extra blankets; warmed if available.
- ☐ For a prone case: the planned face pillow or prone-view head support.
- ☐ For a prone case: compatible flexible breathing-circuit/tube extender.
- ☐ Other positioning aids and pressure-point padding as needed.
- ☐ Patient eye protection/tape, and gloves in your size.

Medication preparation and final supplies

- ☐ Syringes, needles, approved labels, and case-specific medications.
- ☐ Emergency/hemodynamic stock checked: glycopyrrolate, atropine, ephedrine, phenylephrine, epinephrine, labetalol, esmolol, and additional medications per policy.
- ☐ Controlled substances secured and accounted for under facility policy.
- ☐ OG/NG tube, esophageal stethoscope, lubricant, saline, and other procedure-specific supplies when indicated.

Final question: Is this room ready for **this patient, this procedure, and this position**? Review the setup with your preceptor before patient arrival.



Personal verification matters. A technician may complete designated checks, but the anesthesia provider remains responsible for confirming equipment readiness. Learners verify with their preceptor.¹

6

The Anesthesia Machine Check

The single most important principle: there is no universal checkout that applies to every machine. Always follow the manufacturer-specific checkout for the exact workstation in your room, and follow your facility's policy. The 2008 ASA/APSF guidelines define which systems must be evaluated; the manufacturer defines how ([Anesthesia Key](#)).

Background you should know

The 2008 ASA recommendations (developed with the AANA and ASATT) replaced the older 1993 FDA checklist. Rather than one rigid checklist, they describe the systems and subsystems that must be verified, and identify which items may be delegated to a qualified technician versus those the provider must personally verify ([Anesthesia Key](#); [APSF Newsletter Spring 2008](#)).

6A • The 15 Essential Items (ASA/APSF 2008)

Use this as a guide to **what** must be verified. Complete the daily checkout and repeat the required checks before **each case**, following the workstation instructions and local policy.

#	Item	Frequency
1	Verify auxiliary O2 cylinder and self-inflating manual ventilation device are available & functioning	Daily
2	Verify patient suction is adequate to clear the airway	Each case
3	Turn on the delivery system; confirm AC (mains) power is available (not just battery)	Daily
4	Verify availability of required monitors and check alarms	Each case
5	Verify adequate O2 cylinder pressure and valve status per workstation instructions.	Daily
6	Verify piped gas pressures are ≥ 50 psig	Daily
7	Verify vaporizers are filled and filler ports tightly closed	Each case
8	Verify no leaks between flowmeters and the common gas outlet	Daily / after vaporizer change
9	Test scavenging system function	Daily
10	Calibrate the O2 monitor and verify the low-O2 alarm	Daily
11	Verify CO2 absorbent is not exhausted	Each case
12	Breathing system pressure & leak testing	Each case
13	Verify gas flows properly through the circuit on inspiration & exhalation	Each case

1. APSF, pre-anesthesia checkout: <https://www.apsf.org/article/new-guidelines-available-for-pre-anesthesia-checkout/>



6

Checkout Concepts: Not a Universal Sequence

The 15 essential items: continued

- **14. Document completion:** Record the checkout according to local requirements before each case.¹
- **15. Confirm readiness:** Verify ventilator settings and complete the anesthesia-equipment readiness check before each case.¹

Equipment readiness check: Oxygen? Agent, if used? Absorbent? Circuit/leak checks? Monitoring? Alarms? This anesthesia-equipment check does not replace the formal surgical time-out.

6B • What to verify, not a model-independent operating order

Use these categories to understand the checkout. The exact steps, order, tests, acceptable results, and final settings come from the current manufacturer instructions for your workstation and software version, together with local policy.¹

- **Power and backup:** Confirm the required electrical checks and backup ventilation/oxygen resources.
- **Gas supply:** Verify pipelines, connections, and cylinder readiness. Use the specified pressure criteria and return cylinder valves to the required status.
- **Agent delivery:** Check the selected agent, vaporizer installation, fill level, filler closure, and any required leak tests.
- **Breathing and scavenging systems:** Complete the prescribed checks of the circuit, absorber, valves, ventilator, gas flow, and scavenging.
- **Monitoring and alarms:** Perform required calibration and functional tests, confirm alarm limits, and check patient suction.
- **Final configuration:** Set the machine for the planned technique with your preceptor. Do not assume one APL position, selector position, or flow setting is correct for all workstations and techniques.

Do not bypass a failed test. Notify the preceptor, follow the approved troubleshooting/escalation process, and use a verified alternative if needed. An automated pass does not replace the manual checks required by that workstation.¹

1. APSF, equipment-specific pre-anesthesia checkout:
<https://www.apsf.org/article/new-guidelines-available-for-pre-anesthesia-checkout/>



6

Workstation Orientation: Dräger

6C • Use the exact model's instructions. These are orientation prompts, not operational checklists. Locate the current manufacturer instructions and approved local checkout with your preceptor before operating an unfamiliar machine. Do not assume that similarly named machines or different software versions use identical tests.¹

Identify the model in front of you

- **Fabius GS:** Locate its approved pre-use checklist. Ask which checks are automated, which are manual, and how completion is documented.
- **Fabius GS Premium:** Confirm the exact configuration and instructions rather than assuming every Fabius GS step transfers unchanged.
- **Apollo:** Locate the applicable checkout instructions, on-screen prompts, manual checks, and required final configuration.
- **Perseus:** Use its own current instructions and checkout workflow; do not substitute an Apollo sequence.
- **Narkomed GS:** Locate the applicable pre-use procedure and have the preceptor demonstrate the prescribed leak, ventilation, alarm, and scavenging checks.

Ask the same safety questions for each model

- ☐ Where are the approved instructions, and which model/software version do they cover?
- ☐ What must be checked before the first case, before each subsequent case, and after a circuit, vaporizer, or equipment change?
- ☐ Which items are not evaluated by the automated test?
- ☐ What are the acceptable gas-supply pressures, backup-cylinder criteria, and required valve status for this setup?
- ☐ What does a failed test mean, who can troubleshoot it, and what verified backup is available?

Readiness is established by the prescribed checkout and your preceptor's verification, not by recognizing the brand name or completing a generic numbered list.

1. APSF, checkout tailored to the equipment and location:
<https://www.apsf.org/article/new-guidelines-available-for-pre-anesthesia-checkout/>



6

Workstation Orientation: GE and Mindray

Use each entry to locate the correct instructions, not as a substitute for them. Do not apply one model's menu path, test connector, scavenging target, power test, or final settings to another model.¹

- **GE / Datex-Ohmeda Aestiva:** Find the instructions for the exact system and have the preceptor demonstrate its required manual and automated checks.
- **GE Aisys CS2:** Identify the current software and the approved daily/between-case checkout requirements.
- **GE Aespire:** Use the Aespire-specific instructions for the installed configuration; do not substitute an Aisys CS2 sequence.
- **GE Avance:** Locate its own approved test procedure, required manual checks, and final readiness configuration.
- **Mindray A7:** Confirm the applicable operator manual, checkout prompts, and local procedure for this model.
- **Mindray A9:** Use the A9-specific instructions; do not assume all A7 controls, tests, or settings are interchangeable.

For every workstation: Personally verify the required manual checks, suction, backup ventilation/oxygen, agent and absorbent status, monitors, and alarms with the preceptor. Confirm the machine is ready for this patient and technique, and document completion.¹

7

Emergency Meds & Drawing Up Drugs

The following adult reference ranges are for orientation, not automatic orders. Confirm the indication, dose, route, concentration, and patient-specific factors with the preceptor and current formulary. Prepare only the medications needed for the case and rescue plan; label every prepared syringe.

1. APSF, machine-specific checkout principles:
<https://www.apsf.org/article/new-guidelines-available-for-pre-anesthesia-checkout/>



7

Medications for the Case and Rescue

Adult orientation reference only. Individualize the plan and verify every dose, route, concentration, and contraindication with the preceptor and formulary. Not every stocked medication needs to be drawn up.

7A • Induction / hypnotics

Propofol: 1.5–2.5 mg/kg IV induction
Etomidate: 0.2–0.3 mg/kg IV
Ketamine: 1–2 mg/kg IV
Midazolam: 0.02–0.04 mg/kg IV (premedication/amnesia)

Opioids and neuromuscular blockers

Fentanyl: titrate; commonly 1–3 mcg/kg IV
Hydromorphone / morphine: per analgesic plan
Succinylcholine: 1–1.5 mg/kg IV for RSI
Rocuronium: 0.6–1.2 mg/kg IV
Vecuronium / cisatracurium: per case

Reversal: See Section 12 for depth-guided neostigmine/glycopyrrolate and sugammadex dosing, contraindications, and quantitative recovery checks. Paralysis is not anesthesia.

7B • Emergency / hemodynamic medications

Phenylephrine	Pure alpha-agonist; ~50–100 mcg IV bolus for hypotension; may cause reflex bradycardia.
Ephedrine	~5–10 mg IV bolus for hypotension, often with bradycardia.
Epinephrine	Dose and route depend on indication; follow the anaphylaxis, hemodynamic-rescue, or arrest protocol.
Atropine	Adult symptomatic bradycardia: 1 mg IV; repeat every 3–5 minutes; maximum total 3 mg. ³
Glycopyrrolate	0.2 mg IV for selected antisialagogue/bradycardia use; reversal pairing is covered in Section 12.
Vasopressin	Refractory hypotension/vasoplegia per institutional protocol.
Calcium chloride / gluconate	Hyperkalemia, hypocalcemia, or other indicated use; select formulation and dose per protocol.
Naloxone	Opioid reversal; titrate small doses when appropriate, e.g., 40 micrograms IV.
Flumazenil	Benzodiazepine reversal; use with caution and check contraindications.
Labetalol / esmolol	Selected hypertension or tachycardia indications; confirm the drug-specific plan.
Hydralazine	Hypertension; may cause reflex tachycardia. Not a rate-control drug. ³⁴
Nitroglycerin	Selected ischemia or blood-pressure management indications.

3. AHA 2025, adult bradycardia: <https://cpr.heart.org/-/media/CPR-Files/CPR-Guidelines-Files/2025-Accessible/Algorithm-ACLS-Bradycardia-LngDscrp-250725-Ed.pdf>

34. Hydralazine injection labeling:
<https://dailymed.nlm.nih.gov/dailymed/drugInfo.cfm?setid=e31239e0-8c5a-4d10-9404-af32278484a3>



Medication reference, continued

Lidocaine: Antiarrhythmic or case-specific use per protocol.

Dantrolene: Malignant hyperthermia; know the MH cart location (Section 8).

7C • Drug-handling rules

- Label each prepared syringe with drug name, concentration, and preparation details required by policy.²
- Read the label; color alone does not identify a medication. Verify concentration and units.
- Have the planned rescue medications ready before induction; do not draw every stocked drug.
- Keep controlled substances secure. Prepare them when needed without delaying care; document use, returns, and witnessed waste per policy.

8

Know Where the Emergency Equipment Lives

On your first day in each new room and OR suite, physically walk the room with your preceptor and locate every item below. Write the locations in your notebook — in an emergency, you must reach them in seconds.

Find and confirm the location of:

- ☐ GlideScope / video laryngoscope — confirm it's charged and a clean blade is loaded
- ☐ Difficult Airway Cart — know its contents: fiberoptic scope, bougie, LMAs (incl. intubating LMA), cricothyrotomy/surgical airway kit, assorted ETTs and blades ([Difficult Airway, AORN](#))
- ☐ Crash cart / code cart and defibrillator — know how to turn it on and where the pads are
- ☐ Malignant Hyperthermia (MH) cart — dantrolene, sterile water, cold saline, and the MH protocol/hotline number
- ☐ Emergency tracheostomy tray and selection of tracheostomy tubes
- ☐ Backup manual ventilation device (Ambu bag) and backup O2 cylinder
- ☐ Massive transfusion protocol supplies / blood refrigerator and how to activate MTP
- ☐ Rapid infuser / Level 1 and additional fluid warmers
- ☐ Extra suction setup and tubing
- ☐ Emergency drug box / pharmacy Pyxis and the after-hours pharmacy plan
- ☐ Fire extinguisher and OR fire plan; gas shutoff valves
- ☐ How to call for help — the in-room phone, overhead code/STAT page, and who responds

Know the algorithms

- ◆ ASA Difficult Airway Algorithm — have a plan for “cannot intubate, cannot oxygenate.”
- ◆ ACLS / PALS — current protocols.
- ◆ Malignant Hyperthermia protocol — recognition and dantrolene dosing/mixing.
- ◆ Local Anesthetic Systemic Toxicity (LAST) — where the lipid emulsion (Intralipid) is kept.
- ◆ Anaphylaxis — epinephrine first, then airway/fluids/secondary agents.

2. ASA, labeling:

<https://www.asahq.org/standards-and-practice-parameters/statement-on-labeling-of-pharmaceuticals-for-use-in-anesthesiology>



9

Interviewing the Patient

Before the bedside: I review the chart first

I want to know who I am about to meet and which questions need extra attention. Chart review prepares me for the interview; it does not replace speaking with and examining the patient. Review records, pertinent tests, and consultations with your preceptor.⁴

- **Likely airway equipment and ETT size:** I review prior airway documentation and patient factors to anticipate the setup. I confirm the final tube choice and airway plan after my bedside assessment and discussion with the preceptor.
- **Weight:** I check the current weight in kilograms, height, and whether the measurements are recent.
- **Drug allergies:** I review the listed allergies and reactions so I can clarify them with the patient.
- **Medical conditions and medications:** I review the problem list, current treatment, and any recent changes.
- **Prior surgeries:** I look at the procedures, timing, and any documented complications.
- **Smoking, drug use, and alcohol use:** I review the documented history and plan to ask privately and without judgment about current use and the last use.
- **Labs:** I review the relevant results and trends, including any abnormalities that need follow-up.
- **Medical or cardiac assessment:** I read the consultation or “clearance” note, its date, recommendations, and any outstanding concerns. It is not a substitute for today’s anesthesia assessment.
- **Prior studies:** I review available ECG/EKG, echocardiography reports, pulmonary function tests (PFTs), and other relevant studies. I look at the actual findings, not just whether a study was completed.
- **Prior anesthesia records:** I check for difficult mask ventilation or intubation, equipment that worked, and other anesthetic difficulties, reactions, or recovery problems.

My goal: Arrive at the bedside with a focused set of questions, not a blank slate. Review tests that are relevant and available; this list is not a direction to order every test for every patient.

4. ASA, preanesthesia care: <https://www.asahq.org/standards-and-practice-parameters/basic-standards-for-preanesthesia-care>



9

At the Bedside: Verify and Ask

Start with identity and the intended procedure

- **Check the name band:** I ask the patient to state their full name and date of birth, then match both to the identification band and record. I do not rely on a room number or ask only, “Are you Mrs. Smith?” Use the facility’s identification process if the patient cannot respond.⁵
- **Ask in the patient’s own words:** “What procedure are you having today?” Confirm the site and side when applicable and reconcile the answer with the consent and planned procedure. Stop and clarify any mismatch with the team before proceeding.

Then work through the interview

- **NPO status:** “When did you last eat or drink, and what did you have?” Document the actual time and contents, including liquids and any morning medication taken with a sip of water. Clarify uncertainty with the preceptor.
- **Allergies:** Confirm the allergies again and ask what reaction occurred. Compare the answer with the chart rather than assuming the list is complete.
- **New or active symptoms:** Ask about chest pain, shortness of breath (SOB), recent coughs, colds, fever, or other illnesses. Ask what is new or different from the patient’s usual baseline.
- **Smoking, drugs, and alcohol:** Ask about smoking/vaping, recreational drugs, cannabis, alcohol, and relevant prescribed substances. Clarify amount, frequency, and last use; share this information with the preceptor because it may change the anesthetic plan.
- **Prior anesthesia:** “Have you ever had problems with anesthesia?” Clarify any reported difficult airway, reaction, awareness, prolonged recovery, or severe nausea/vomiting and compare it with prior records.
- **Medical conditions and morning medications:** Review the conditions with the patient, not just the problem list. Ask which medications were actually taken, the dose and time, and which were held. Follow up on diabetes, cardiac, pulmonary, and renal issues as outlined next.⁴

Do not quietly move on from a concern. New chest pain, worsening breathing, a procedure mismatch, or an unresolved medication or fasting issue needs prompt review with the preceptor. A student does not independently decide to proceed or cancel.

5. Joint Commission, two identifiers: <https://www.jointcommission.org/standards/standard-faqs/critical-access-hospital/national-patient-safety-goals-npsg/000001545/>

4. ASA, preanesthesia care: <https://www.asahq.org/standards-and-practice-parameters/basic-standards-for-preanesthesia-care>



9

Diabetes and Medication Follow-Up

For a patient with diabetes, I connect the chart findings with what actually happened at home this morning. Medication reconciliation includes drugs prescribed for weight management, heart failure, or kidney disease, not only drugs the patient calls “diabetes medicines.”

Check glucose control and recent treatment

- **HbA1c and morning glucose:** Review the most recent HbA1c and its date, then check the current point-of-care glucose according to policy. HbA1c describes longer-term control; it does not replace today’s glucose measurement. Report abnormal results and symptoms to the preceptor.⁶
- **Insulin and other medications:** Ask the exact drug, dose, and last administration time, including insulin given overnight or this morning. Identify pumps or continuous glucose monitors and confirm the perioperative plan. In type 1 diabetes, basal insulin must not simply be omitted because the patient is NPO; the regimen requires an individualized plan.⁶

Specifically ask about GLP-1 and SGLT2 drugs

- **GLP-1 or dual GIP/GLP-1 therapy:** Ask the medication name, dose, last dose, recent initiation or dose increases, and nausea, vomiting, bloating, constipation, or other symptoms of delayed gastric emptying. These drugs are not an automatic contraindication to anesthesia.^{6,7}
- **GLP-1 risk assessment:** Most low-risk patients may continue therapy under current multisociety guidance. Higher-risk patients may need a 24-hour liquid-only diet, gastric ultrasound by a qualified clinician, an adjusted anesthetic approach, or delay. Confirm the team’s instructions and what the patient actually followed; do not impose a blanket hold or clearance rule.^{6,7}
- **SGLT2 inhibitors:** Verify that the prescribed preoperative hold was completed. For elective surgery, usual U.S. guidance is at least 3 days for canagliflozin, dapagliflozin, or empagliflozin, and 4 days for ertugliflozin. These drugs can cause perioperative euglycemic ketoacidosis, even without marked hyperglycemia.⁶

If the medication was not held, the history is uncertain, or the patient is unwell: Notify the preceptor and follow the local assessment pathway. A normal glucose alone does not exclude SGLT2-associated ketoacidosis. Urgent cases require an individualized plan rather than simply applying elective-surgery hold times.⁶

6. ADA, Standards of Care 2026: <https://pmc.ncbi.nlm.nih.gov/articles/PMC12690180/>

7. ASA, multisociety GLP-1 guidance:

<https://www.asahq.org/about-asa/newsroom/news-releases/2024/10/new-multi-society-glp-1-guidance>



9

Cardiac and Pulmonary Follow-Up

Cardiac history: clarify stability and medications

- **History and current symptoms:** Ask about coronary disease, prior myocardial infarction, recent stents or other cardiac interventions, arrhythmias, and cardiac devices. Clarify chest pain, exercise tolerance, shortness of breath, palpitations, or syncope and report new or worsening findings.⁸
- **Recent stents/interventions:** Confirm what was done and when. Review the documented antiplatelet or anticoagulant plan and actual last doses. Do not independently tell a patient to stop these medications; stent timing and interruption of antiplatelet therapy can change procedural risk and timing.⁸
- **Morning medications and ACE inhibitors/ARBs:** Ask exactly what was taken. Continuation or withholding depends on the indication and procedure; management for hypertension may differ from heart failure. If an ACE inhibitor or ARB was taken, alert the preceptor and confirm the hypotension-rescue plan, including access to vasopressin for refractory vasodilatory hypotension. This does not mean giving vasopressin automatically or prophylactically.^{8,9}
- **Congestive heart failure (CHF):** I look for edema and ask about increased swelling, recent weight gain, difficulty lying flat, worsening dyspnea, and recent admissions. Review relevant echo findings and medications. New or worsening symptoms need reassessment, even if an earlier “clearance” note is present.⁸

Pulmonary history: ask, listen, and compare with baseline

- **Asthma/COPD and inhaler use:** Ask how often albuterol or another rescue inhaler is needed, when it was last used, whether use has increased, and about recent exacerbations, steroid treatment, hospitalizations, or intubations. More frequent rescue use may signal poor control and warrants discussion with the preceptor.¹⁰
- **Breath sounds and active wheezing:** Listen to the lungs and assess the patient’s current respiratory status. Wheezing may require a prescribed bronchodilator/respiratory treatment and reassessment before anesthesia. Poorly controlled disease may warrant postponing elective surgery; a treatment does not by itself establish readiness.¹⁰

8. ACC/AHA guideline overview: <https://www.ccm.org/content/92/4/213>

9. APSF, RAS antagonists and vasoplegia:
<https://www.apsf.org/article/vasoplegic-syndrome-and-renin-angiotensin-system-antagonists/>

10. OpenAnesthesia, asthma: <https://www.openanesthesia.org/keywords/anesthesia-for-patients-with-asthma/>



9

Renal, Airway, and IV Assessment

Renal disease: make a deliberate fluid plan

- **Dialysis and volume status:** Ask about dialysis type, last treatment, usual schedule, recent problems, urine output, and fluid restrictions. Review relevant electrolytes, particularly potassium, and assess for overload or depletion. Protect a fistula/graft arm from IV placement and blood pressure cuffs.¹¹
- **My smaller-bag precaution:** When limiting unnecessary fluid is part of the plan, I may consider hanging a 500 mL bag rather than a 1 L bag. For example, a 500 mL normal-saline bag may be used **if normal saline is the selected fluid**. The bag is a reminder to track delivery, not an order to infuse the whole volume.
- **Individualize, do not simply restrict:** Confirm fluid type, rate, boluses, and monitoring with the preceptor. Renal patients can be volume overloaded or hypovolemic, especially after dialysis. Avoid both fluid overload and inadequate perfusion; renal disease alone does not mandate normal saline or a fixed fluid limit.¹¹

Airway and teeth: examine, do not assume

- **Focused airway assessment:** I assess Mallampati class, thyromental distance (TMD), mouth opening, neck movement, and dentition. I compare the exam with prior airway records and confirm the equipment, likely ETT size, and backup plan with the preceptor.⁴
- **Dental condition:** Ask specifically about loose, cracked, or missing teeth, dentures or removable appliances, and extensive dental work such as crowns, bridges, or implants. Document concerns and alert the preceptor so the airway approach accounts for fragile dental work.

Before leaving pre-op: personally verify the IV

- **Trace and inspect the line:** Check the IV site, dressing, connections, and tubing. Ask about pain or burning and look for swelling, leakage, or other signs of infiltration. Confirm patency using the approved flush/assessment process while observing the site; flow in the tubing alone is not enough.¹²
- **If the IV is questionable:** Do not force a flush or give anesthesia drugs through a suspect line. Stop the infusion if infiltration/extravasation is suspected, notify the preceptor, follow the facility's management protocol, and arrange reliable access before proceeding.¹²

Practical reminder: The last thing you need as a new student is to discover an infiltrated IV after bringing the patient into the OR. Verify it in pre-op, then report your findings and unresolved concerns to the preceptor.

11. OpenAnesthesia, renal replacement therapy: <https://www.openanesthesia.org/keywords/renal-replacement-therapy-anesthetic-considerations/>

4. ASA, preanesthesia care: <https://www.asahq.org/standards-and-practice-parameters/basic-standards-for-preanesthesia-care>

12. Ausmed, peripheral IV assessment: <https://www.ausmed.com/learn/articles/intravenous-cannula>



10

Explaining the Anesthetic Plan

Patients have usually never met you before. Introduce your role, speak calmly, and build trust quickly. Explain the actual plan in everyday language, invite questions, and review your explanation with the preceptor before presenting it.

A patient-friendly example for a planned asleep induction

- **What general anesthesia means:** “The plan is general anesthesia. We will give medicine through your IV to make you fully asleep, or unconscious, for the procedure. We will adjust the anesthetic throughout your surgery.”¹³
- **What happens first:** “In the operating room, we will help you onto the bed, connect monitors, and ask you to breathe oxygen through a mask. Then we will give the medicines that start the anesthetic.”
- **How we support breathing:** “For your procedure, we plan to use [name the planned device]. An LMA sits in the back of the throat above the voice box. An endotracheal tube, or ETT, goes into the windpipe. The device helps us keep your airway open and support breathing.”
- **When the airway goes in and comes out:** “For this planned asleep approach, we place the breathing device after you are asleep. We remove it when it is safe as you recover. Many people do not remember the removal, but some do; we will help you through that part.” Do not promise that removal can never be remembered.
- **Airway-related risks:** “You may have a sore throat afterward. There is also a risk of injury to the lips, mouth, or teeth, including chipping or cracking a tooth. Please tell us about loose teeth, crowns, bridges, implants, or other dental work.”
- **Continuous care:** “A qualified anesthesia professional will be with you throughout the anesthetic. We watch your heart rate, blood pressure, oxygen level, breathing, exhaled carbon dioxide, and the anesthetic being delivered, and make adjustments as needed.”¹⁴

Tailor the explanation: This example is not the full informed-consent discussion. Explain the planned technique, relevant alternatives, patient-specific risks, and the possibility of changes. If an awake airway approach is planned, explain it honestly rather than saying the airway will be placed after the patient is asleep.

13. ASA, general anesthesia: <https://madeforthismoment.asahq.org/anesthesia-101/types-of-anesthesia/general-anesthesia/>

14. ASA, basic monitoring: <https://www.asahq.org/standards-and-practice-parameters/standards-for-basic-anesthetic-monitoring>



10

Comfort, Recovery, and Trust

Explain pain and nausea management

“We will plan medicines and other treatments to help manage pain and reduce nausea. After surgery, the recovery-room team will assess how you feel. We will make sure there are appropriate orders so they can treat further pain or nausea if needed.”¹⁵

- **Set a realistic expectation:** Explain prevention and treatment without promising that the patient will have no pain, no nausea, or no other side effects. Confirm the actual analgesic and antiemetic plan and PACU orders with the preceptor.

Describe recovery as a process, not a stopwatch

“After surgery, you will go to the post-anesthesia care unit, or PACU, while you wake up. Everyone responds to and clears medicines differently. We will watch your breathing, circulation, alertness, comfort, and nausea before deciding you are ready for the next stage of care.”¹⁵

- **A local estimate, not a promise:** If reasonable for this patient and facility, say, “For a straightforward recovery here, the initial phase may be about 45 minutes to an hour, but it can be shorter or longer. We move you when you meet the recovery criteria, not simply when that amount of time has passed.”
- **Where family fits:** “When you are ready, you may return to your hospital room or move to the ambulatory recovery area to see your family, depending on the procedure and facility policy.” Do not promise an exact reunion or discharge time; some patients need longer observation or a different destination.¹⁵

Leave room for the patient’s questions

- **Ask openly:** “What questions or worries do you have?” Pause long enough for an answer. Check understanding rather than rushing into the next task.
- **Be professional and reassuring:** Use a calm tone and clear explanations. Confidence means being prepared and honest, not pretending to know an answer you do not know.
- **Use your team:** If a question is outside your knowledge, say, “That is an important question. Let me bring my preceptor into the conversation so we can answer it accurately.” Complete the required consent discussion before sedating medications.

15. ASA, recovery after anesthesia: <https://madeforthismoment.asahq.org/preparing-for-surgery/recovery/>



11

Bringing Your First Patient to the OR

You will be nervous, and that is normal. You are not expected to manage your first induction alone. Agree on the plan, your role, and when your preceptor will take over. This walkthrough is for a supervised, routine adult IV induction; it is not an awake-airway, rapid-sequence, pediatric, or emergency protocol.

Before any premedication or transfer

- **Confirm with the circulating RN and preceptor:** Required surgical and anesthesia consent processes are complete; forms are signed as required; the surgeon has seen the patient; site marking and the final preoperative checks are complete. Resolve unanswered questions first.
- **Midazolam (Versed), if appropriate:** Give it only after your preceptor confirms the indication, dose, consent/readiness status, and monitoring/transport plan. Do not give a sedative while the team is still trying to obtain consent or essential patient decisions.
- **If sedation was given too early:** Stop and notify the responsible clinicians. Sedation can impair decision-making, but it does not automatically eliminate capacity or mean every case must be cancelled. Follow the facility's capacity, consent, and escalation process.¹⁶

Move safely and capture the correct times

- **Maintain observation during transport:** Use monitoring, oxygen, assistance, and safety measures appropriate to the patient and medication effects. Do not leave a sedated patient unattended.
- **Record room entry:** Note the actual "in-room" time. It is a separate event from anesthesia start. CMS defines anesthesia time as beginning when the anesthesia practitioner starts preparing the patient for anesthesia in the OR or equivalent area; follow the facility and payer rules rather than automatically equating it with room entry.¹⁷
- **Protect the IV:** Hang the fluid on the IV pole and trace the tubing. Make sure it has enough slack and is not caught on the stretcher, bed rail, or equipment.
- **Lock before transfer:** Position the stretcher/cart beside the OR table and verify that both are locked and appropriately aligned. Use the required staff and transfer aid, protect lines and limbs, and do not ask a sedated patient to move without appropriate assistance.

16. UW Medicine, capacity and consent: https://www.uwmedicine.org/sites/stevie/files/2019-11/20190604_ICM.pdf

17. CMS, anesthesia time: <https://www.cms.gov/files/document/02-chapter2-ncci-medicare-policy-manual-2026-final.pdf>



11

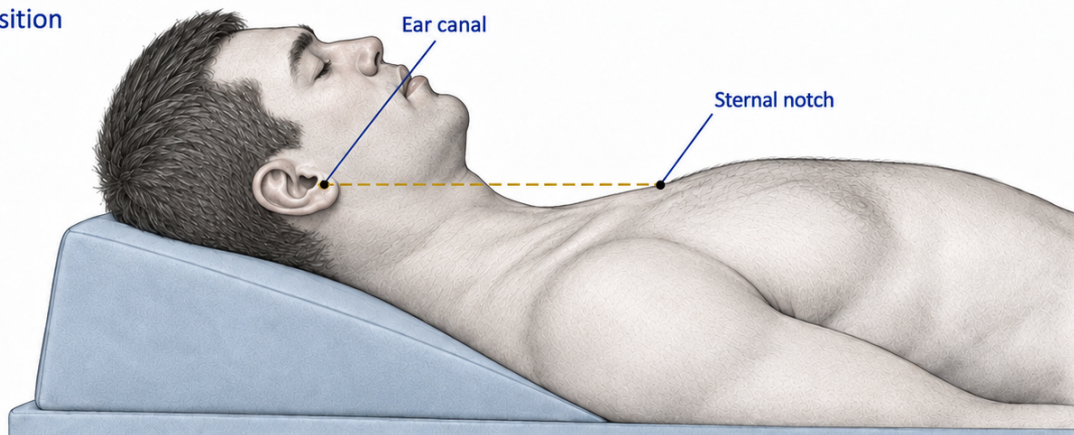
Position Before You Induce

Review anatomy, mobility, airway assessment, and the intended device with your preceptor. Position the patient before induction; no single setup fits everyone.¹⁸

Sniffing position



Ramped position



Sniffing: Support the occiput to flex the lower neck, with gentle head extension when safe. Adjust the support to the patient; do not force the neck or use a fixed pillow height.¹⁸

Ramping: Support the upper torso and head to align the external auditory meatus (ear canal) horizontally with the sternal notch. Confirm the position with the preceptor.¹⁸

Cervical-spine precautions: Injury or restricted movement requires a modified plan. Do not routinely extend the head in these patients. Illustration is schematic, not to scale.¹⁸

18. MSD Manual, positioning guidance; original schematic, not to scale: <https://www.msmanuals.com/professional/critical-care-medicine/how-to-do-other-airway-procedures/how-to-do-oro-tracheal-intubation-using-video-laryngoscopy>



11

Monitors and Preoxygenation

Apply monitors promptly and obtain a baseline

- **My usual sequence:** I start the blood pressure cycle first because it takes time to obtain a reading, then apply the pulse oximeter and ECG leads. Apply them together when help is available; do not let this preferred sequence delay oxygenation monitoring or a necessary baseline assessment.
- **Verify signals, not just connections:** Confirm a reliable BP, pulse-oximetry signal, and ECG tracing. Check the patient as well as the numbers. Required monitoring and alarms must be active before induction; monitor a sedated patient appropriately before this point, too.¹⁴

Preoxygenate using the agreed technique

- **Explain the mask:** Reassure the patient that the oxygen mask is part of preparation. Confirm oxygen delivery and a good mask seal, and assess breathing and oxygenation. Use the preceptor's patient-specific preoxygenation target; a satisfactory SpO₂ reading alone is not the entire readiness assessment.
- **Set the APL valve for the ventilation mode:** The adjustable pressure-limiting valve is commonly set to open/Spont for unassisted spontaneous breathing in a circle system. For manual positive-pressure ventilation, adjust it to permit controlled ventilation while limiting pressure. Follow the exact machine instructions; CPAP/PEEP or other techniques may need different settings.¹⁹
- **Opioid timing:** If an opioid is part of the agreed plan, give it at the time your preceptor directs so its effect can help blunt laryngoscopy. It is not mandatory in every induction. Watch ventilation and hemodynamics closely and have support ready.
- **IV readiness:** Reconfirm patency and drug delivery access. Use the prescribed fluid rate or bolus; do not automatically run fluids wide open, especially when cardiac or renal disease or another condition changes fluid tolerance.

Final scan before induction: Suction tested and reachable; airway and backup equipment ready; IV patent; oxygen delivery confirmed; ECG, blood pressure, and SpO₂ readings acceptable for this patient; alarms active; rescue drugs available; preceptor present and ready. State any concern before giving drugs.¹⁴

14. ASA, basic monitoring: <https://www.asahq.org/standards-and-practice-parameters/standards-for-basic-anesthetic-monitoring>

19. Dräger Apollo, APL example; use the local workstation manual:
https://www.draeger.com/Content/Documents/Products/IfU_Apollo_SW_3.n_EN_US_9038237.pdf



11

Induction: One Step at a Time

Agree on the drug sequence, doses, anticipated hemodynamic effects, airway strategy, and backup plan before beginning. Use closed-loop communication. Your preceptor directs the induction and remains ready to assist.

Follow the agreed drug and airway sequence

- **Confirm the planned adjuncts:** Check whether the opioid has been given, if it is part of the plan. Lidocaine may be used to reduce propofol injection discomfort when appropriate; confirm rather than treating it as a required medication.
- **Give the selected induction agent:** Administer it under the preceptor's direction and assess its clinical effect. Loss of response to verbal command and the eyelash reflex may be observations used in that assessment, but neither is a stand-alone guarantee of adequate anesthesia for airway manipulation or surgery.
- **Neuromuscular blockade, when indicated:** Give it only as directed after adequate hypnosis is established, while maintaining anesthetic delivery. Paralysis does not provide unconsciousness or analgesia. An LMA often does not require paralysis for insertion, but blockade may still be indicated for a particular operation. Its use depends on the airway and surgical requirements.²⁰
- **Allow adequate drug effect:** Rocuronium onset depends on dose, circulation, and the patient. Assess readiness with the preceptor and appropriate neuromuscular monitoring rather than relying on a fixed countdown.²⁰

Maintain oxygenation and ask for help early

- **Mask ventilation:** In the routine induction pathway where mask ventilation is planned, provide controlled ventilation while waiting for suitable intubating conditions. Use a good seal, appropriate airway-opening maneuvers, chest movement, and a capnography waveform to assess ventilation. An oral airway or two-person technique may be needed.¹⁴
- **If ventilation is uncertain or difficult:** Say so immediately and ask the preceptor to help or take over. Do not continue repeated attempts while oxygenation deteriorates. The airway-rescue pathway takes priority over completing this written sequence.
- **Eye protection:** Gently close and protect the eyes promptly after loss of consciousness, commonly before airway manipulation when feasible. Follow the preceptor's technique and local policy, avoid pressure on the eyes, and do not delay urgent oxygenation to tape them.²¹

20. FDA, rocuronium prescribing information: https://www.accessdata.fda.gov/drugsatfda_docs/label/2025/219108s000lbl.pdf

14. ASA, basic monitoring: <https://www.asahq.org/standards-and-practice-parameters/standards-for-basic-anesthetic-monitoring>

21. Eye protection review: <https://pmc.ncbi.nlm.nih.gov/articles/PMC5062235/>



11

Confirm and Secure the Airway

After an endotracheal tube is placed

- **Connect and ventilate:** With the preceptor, remove the stylet when used, inflate the ETT cuff to achieve an appropriate seal, and check cuff pressure according to device instructions and local practice. Remove the face mask from the circuit, connect the tube, and ventilate.
- **Confirm placement:** Look for sustained exhaled-CO₂ waveforms over repeated breaths, chest movement, appropriate ventilation, and bilateral breath sounds as assessed by the team. Capnography is essential; chest rise or auscultation alone is not enough. An absent or unexpected waveform requires immediate evaluation and help.¹⁴
- **Check depth and document it:** Note the actual centimeter marking at the lip or teeth and the reference point. Depth is patient-specific and must be verified, including after repositioning. ETT diameter does not determine the correct insertion depth.
- **Secure and recheck:** Keep control of the airway while it is secured with the approved method. Recheck circuit connections, capnography, ventilation, and tube position after fixation.

After an LMA or another supraglottic device is placed

- **Use that device's instructions:** Positioning, cuff inflation or absence of an inflatable cuff, fixation, and insertion markers differ among devices. If a cuff is present, follow its pressure limits; do not use an ETT cuff routine for every supraglottic airway.²²
- **Confirm ventilation and seal:** Check sustained capnography, chest movement, adequate ventilation, and an acceptable seal. A sensation of the device seating, resistance, or a “pop” does not establish correct placement.^{14,22}
- **Document appropriately:** Record the device type, size, placement checks, and any device-specific position/markings required locally. Do not apply a universal centimeter-at-the-lip rule to all LMAs.²²

Do not let airway success become a distraction. Keep ventilation and anesthetic delivery continuous while confirming, securing, and documenting the device. If anything is uncertain, stop and ask the preceptor.

14. ASA, airway confirmation and capnography:
<https://www.asahq.org/standards-and-practice-parameters/standards-for-basic-anesthetic-monitoring>

22. Teleflex LMA Supreme example; use the actual device instructions: https://www.teleflex.com/la/en/product-areas/anesthesia/airway-management/lma-airways/product-literature/AN_LMA_Supreme_User_Guide_MC-000163_Rev1.pdf



11

The Immediate Post-Airway Scan

VERBAL CHECK: “VENTILATION AND ANESTHETIC DELIVERY CONFIRMED.”

After placing the airway, deliberately verify both. Do not assume that a connected circuit means the ventilator is running, or that an induction dose will maintain anesthesia for the case.

Deliberately establish the maintenance plan

- **Ventilation:** If controlled ventilation is planned, select the prescribed mode, turn it on, and confirm that breaths are actually delivered. Check chest movement, exhaled volumes, airway pressures, capnography, oxygen delivery, and alarms. Some LMA techniques use spontaneous or assisted ventilation; verify the intended mode rather than automatically applying one setting to everyone.¹⁴
- **Volatile anesthesia:** If part of the plan, turn on the selected agent and confirm its inspired and end-tidal concentrations, not just the dial position. For total intravenous anesthesia, verify the infusion, pump settings, IV connection, and patency instead. “Turn on the gas” is not a universal instruction for TIVA.
- **Airway securement:** Confirm that the ETT or LMA is secured appropriately and that moving to mechanical ventilation has not displaced it or disconnected the circuit. Maintain manual support of the device until fixation is complete.
- **Recheck blood pressure:** Obtain a prompt post-induction reading and reassess frequently during instability. Induction may cause hypotension; identify the likely cause with the preceptor and treat with the appropriate medication, fluid strategy, or other intervention rather than automatically giving fluid.¹⁴
- **Eyes, temperature, and warming:** Confirm eye protection is in place. Add the appropriate temperature probe and warming device when indicated, or verify them if already placed. Avoid unnecessary delay in temperature management.^{14,21}

Then scan the whole patient and system again

Airway position and security; oxygenation; ventilation and ETCO_2 ; blood pressure and rhythm; anesthetic delivery; IV access; fluids and infusions; temperature; pressure points; and the next anticipated event. Make ventilator adjustments with your preceptor to meet the patient’s oxygenation and ventilation goals.

A useful habit: Say the important checks aloud. It gives the preceptor a clear picture of what you have verified and helps prevent a task from being overlooked while you are excited or overwhelmed.

14. ASA, ongoing monitoring:
<https://www.asahq.org/standards-and-practice-parameters/standards-for-basic-anesthetic-monitoring>

21. Eye protection review: <https://pmc.ncbi.nlm.nih.gov/articles/PMC5062235/>



11

Before Incision: Think Ahead

Antibiotics and surgical readiness

- **Confirm prophylaxis:** Does this procedure require antibiotics, and have they already been given? Verify the selected agent, allergy considerations, dose, administration time, and redosing plan under local policy. Do not assume someone else has given them.
- **Time them for incision:** Standard prophylaxis guidance generally starts antibiotics within 60 minutes before incision, with up to 120 minutes for vancomycin or fluoroquinolones because of infusion time. Follow the agent-specific protocol and coordinate with the circulating RN and surgeon; the formal surgical time-out is still required.²³

Antiemetics: match timing to the medication

- **My usual timing:** When included in the plan, I generally give dexamethasone (Decadron) near induction and ondansetron (Zofran) toward the end of surgery. Their recommended prophylactic timing differs; think about when the effect is needed, not just when it is convenient to chart.²⁴
- **Individualize the regimen:** Review PONV risk, contraindications, medications already given, and the rescue plan. Do not automatically give or repeat an antiemetic without checking the patient's plan and the record.

Confirm adequate anesthesia before stimulation

- **Prepare for incision:** Tell your preceptor when incision is approaching. Confirm anesthetic and analgesic delivery, airway and ventilation, and hemodynamic readiness before the surgeon begins.
- **For volatile anesthesia:** Review the age-adjusted end-tidal concentration/MAC estimate with the preceptor. MAC is a measure of potency and immobility, not a stand-alone guarantee of unconsciousness or analgesia. It is not a TIVA target.²⁵
- **Balance anesthesia and circulation:** Hypotension is not proof of adequate anesthetic depth, and paralysis is not anesthesia. Treat hemodynamic problems while ensuring the patient continues to receive an appropriate anesthetic. This judgment develops with supervised experience.^{20,25}

23. ASHP, prophylaxis timing: <https://guideline.guidelinecentral.com/i/109326-ashp-surgical-prophylaxis-guidelines/13>

24. Fifth PONV consensus, supplemental timing table:

https://cdn-links.lww.com/permalink/aa/f/aa_1_1_2025_09_14_gan_aa-d-25-00465_sdc3.pdf

25. StatPearls, MAC: <https://www.ncbi.nlm.nih.gov/books/NBK532974/>

20. FDA, rocuronium: https://www.accessdata.fda.gov/drugsatfda_docs/label/2025/219108s000lbl.pdf



11

Documentation and Ongoing Care

Learn the record with your preceptor

Every facility is different. Some still use paper anesthesia records; others use electronic systems. The first day can feel overwhelming, and you are not expected to know a new charting system immediately.

My teaching approach: I initially have students focus on the patient while I handle the charting. Once I understand the student's capabilities and they are becoming comfortable with the clinical flow, I introduce documentation. You will learn the system with support, not by trying to do everything at once.

- **Agree on responsibilities:** Know who is documenting during induction and who is continuously watching the patient. As you begin charting, confirm supervision and avoid becoming absorbed in the screen.¹⁴
- **Learn the required entries:** With the preceptor, document the assessment, relevant times, medications and doses, fluids, airway device and verified position, ventilation, vital signs, events, interventions, and responses according to facility requirements. An electronic import does not replace checking accuracy.
- **Record actual events:** Distinguish room-entry, anesthesia-start, induction, airway, and other required times. Do not substitute one timestamp for another or guess values that were not observed. Follow the correction or late-entry policy when necessary.¹⁷

Stay engaged throughout the case

- **Monitor trends, not isolated numbers:** Continue watching oxygenation, ventilation, circulation, temperature as indicated, and anesthetic delivery. Reassess after interventions, position changes, and changes in surgical stimulation.¹⁴
- **Anticipate the next event:** Ask which parts of this procedure are particularly stimulating and what changes are expected. Prepare for the next step rather than waiting until it has already happened.
- **Keep communicating:** Report changes and concerns early. There are many procedures and many patient-specific variations; sound habits and repeated supervised practice build confidence.

Be patient with yourself. You do not need to know everything on your first day. Be prepared, be attentive, ask for help, and keep learning. You can do this.

14. ASA, continuous presence and monitoring:

<https://www.asahq.org/standards-and-practice-parameters/standards-for-basic-anesthetic-monitoring>

17. CMS, anesthesia time: <https://www.cms.gov/files/document/02-chapter2-ncci-medicare-policy-manual-2026-final.pdf>



12

Emergence: Prepare Before the Last Stitch

Knowing when to turn off the gas is an art that develops with experience. Even after years of practice, some patients wake sooner than expected and others take longer. Every patient is different. Watch the patient, the surgical progress, and the anesthetic trends with your preceptor rather than following a fixed countdown.

Build the emergence plan together

- **Ask what is left:** Confirm the remaining surgical stimulation, dressings, positioning, and any additional procedures. Discuss the intended extubation approach, expected airway risks, postoperative destination, and rescue plan before reducing anesthesia.²⁹
- **Check suction and rescue equipment:** Suction must work and be within reach. Have the face mask, oxygen, airway adjuncts, and equipment for assisted ventilation and reintubation immediately available. Your preceptor should be present and ready.²⁹
- **Review comfort and nausea prevention:** Reassess analgesia and give ondansetron (Zofran) if appropriate to the plan, checking prior doses, contraindications, and timing. Do not duplicate an antiemetic simply because the case is ending.²⁴
- **Prepare for reversal:** Verify the neuromuscular blocker, dose history, current block depth, and reversal plan. Have the nerve stimulator/quantitative monitor in place and working. Reversal is guided by measured recovery, not elapsed case time alone.²⁶
- **Remove devices thoughtfully:** Obtain and document the final temperature as needed, then remove the esophageal/oral temperature probe before airway removal. Remove skin probes when no longer needed. Gently remove eye tape at the appropriate time, avoiding eye pressure; keep the eyes protected while the patient remains anesthetized.

Reduce the anesthetic without rushing the patient

- **Volatile anesthetic:** Coordinate reducing and turning off the vaporizer with the preceptor. When ready for emergence, increase fresh-gas oxygen flow as appropriate to the workstation and plan to promote washout. Follow inspired/end-tidal agent levels, oxygenation, ventilation, and clinical recovery. For TIVA, adjust the infusion instead; there may be no volatile agent to wash out.

Keep supporting the patient. Low agent concentration or a vaporizer dial set to OFF is not permission to extubate. Maintain oxygenation, ventilation, and appropriate anesthesia until the intended emergence stage and readiness criteria are met.²⁹

29. BJA Education, safe extubation: <https://pmc.ncbi.nlm.nih.gov/articles/PMC8606605/>

26. ASA, neuromuscular recovery: <https://csa-online.org/wp-content/uploads/2023/02/NMB23.pdf>

24. PONV consensus guidance: https://cdn-links.lww.com/permalink/aa/f/aa_1_1_2025_09_14_gan_aa-d-25-00465_sdc3.pdf



12

Reversal: Measure Recovery, Not Just Twitches

Before extubation after neuromuscular blockade: Confirm a quantitative train-of-four (TOF) ratio ≥ 0.9 at the **adductor pollicis**. Four visible twitches, no visible fade, a hand squeeze, or a head lift cannot by themselves establish full recovery.²⁶

Choose the agent for the drug and measured depth

- **Quantitative monitoring:** Check correct electrode/sensor placement and device function. Use the ulnar nerve/adductor pollicis for recovery assessment; follow the device instructions. A qualitative nerve stimulator is useful, but it does not display the numerical TOF ratio needed to exclude residual block.²⁶
- **Rocuronium or vecuronium:** ASA guidance favors sugammadex over neostigmine for deep, moderate, or shallow block. Neostigmine is a reasonable alternative at minimal block, a quantitative TOF ratio of 0.4 to less than 0.9.²⁶
- **Other blockers and spontaneous recovery:** Sugammadex does not reverse cisatracurium, atracurium, or succinylcholine. Select an appropriate recovery/reversal plan. If spontaneous recovery to TOF ratio ≥ 0.9 is objectively confirmed, pharmacologic reversal is not automatically required.^{26,27}

Neostigmine + glycopyrrolate: adult teaching reference

- **Neostigmine:** At minimal block, ASA guidance limits the dose to **40 micrograms/kg IV (0.04 mg/kg)**; when the TOF ratio is above 0.6 but below 0.9, **15–30 micrograms/kg** is usually adequate. These are depth-guided recommendations, not a fixed dose. Also respect the labeled **5 mg total maximum**; lower depth-based limits still apply. Verify with the preceptor.^{26,33}
- **Glycopyrrolate:** Pair **0.2 mg IV for each 1 mg of neostigmine** to limit muscarinic effects. For example, 3 mg of neostigmine pairs with 0.6 mg of glycopyrrolate; this is a ratio calculation, not a standard patient dose. Give the antimuscarinic before or with neostigmine, following the product instructions; give it first if the patient is bradycardic.^{28,33}
- **Allow recovery and remeasure:** Continue ventilatory support and monitoring. Do not give neostigmine at deep block and expect reliable reversal, or repeat it blindly. Do not extubate based only on a waiting interval or the fact that reversal was administered.²⁶

26. ASA 2023, quantitative monitoring and depth-guided reversal: <https://csa-online.org/wp-content/uploads/2023/02/NMB23.pdf>

27. Sugammadex labeling:

<https://dailymed.nlm.nih.gov/dailymed/fda/fdaDrugXsl.cfm?setid=5b25f16e-726a-4464-88da-b188c2f4e038&type=display>

28. FDA, glycopyrrolate labeling: https://www.accessdata.fda.gov/drugsatfda_docs/label/2025/214919s002lbl.pdf

33. Neostigmine labeling:

<https://dailymed.nlm.nih.gov/dailymed/fda/fdaDrugXsl.cfm?setid=b0b509c3-8dc8-48cc-b7a8-16b93ad706c6&type=display>



12

Sugammadex: Match Dose to Block Depth

Adult IV reference for rocuronium- or vecuronium-induced blockade. Dose using **actual body weight** and measured block depth. Verify the medication, concentration, calculation, contraindications, and plan with your preceptor before administration.²⁷

2 mg/kg IV

Routine reversal when the **second twitch (T2) has reappeared** during TOF stimulation after rocuronium or vecuronium.²⁷

4 mg/kg IV

Deep block with **1–2 post-tetanic counts (PTC) and no TOF responses** after rocuronium or vecuronium. Do not substitute a smaller fixed dose without considering the measured depth.²⁷

16 mg/kg IV: a specific immediate-reversal indication

For a clinical need to reverse block approximately **3 minutes after a single rocuronium dose of 1.2 mg/kg**. This is not the routine emergence dose and is not the labeled immediate-reversal regimen for vecuronium. Reversal does not replace emergency airway management.²⁷

Safety checks that remain essential

- **Confirm recovery:** Continue ventilation until adequate spontaneous breathing and airway maintenance are established. Recheck the quantitative TOF ratio; confirm ≥ 0.9 before extubation. Sugammadex does not reverse opioid or sedative respiratory depression.^{26,27}
- **Renal function:** Labeling does not recommend use when creatinine clearance is less than 30 mL/min, including dialysis. Discuss an alternative or specialist-directed plan rather than treating this as routine dosing.²⁷
- **Watch closely:** Be prepared for marked bradycardia, including cardiac arrest, and hypersensitivity/anaphylaxis. Underdosing can permit recurrent block. Re-paralysis after sugammadex requires a specific drug/timing plan.²⁷
- **Discharge counseling:** Patients using hormonal contraception need an additional nonhormonal/backup method for 7 days after sugammadex; communicate this to the recovery/discharge team and document counseling.²⁷

27. Sugammadex prescribing information:

<https://dailymed.nlm.nih.gov/dailymed/fda/fdaDrugXsl.cfm?setid=5b25f16e-726a-4464-88da-b188c2f4e038&type=display>

26. ASA, neuromuscular recovery: <https://csa-online.org/wp-content/uploads/2023/02/NMB23.pdf>



12

Awake Extubation: Readiness Comes First

For a routine adult awake extubation, review all of the following with your preceptor. These are readiness domains, not a stand-alone protocol. A difficult airway, aspiration risk, airway swelling, OSA, or significant cardiopulmonary disease may require a different strategy or delayed extubation.²⁹

Confirm readiness before removing the ETT

- ☐ **Awake and protecting the airway:** The patient is appropriately responsive, follows commands when able at baseline, and can maintain airway patency and protective reflexes. Secretions are manageable and the cough is effective.²⁹
- ☐ **Adequate breathing and oxygenation:** Regular spontaneous breathing, adequate tidal/minute ventilation, and acceptable CO₂ and SpO₂ for the patient without excessive ventilatory support. Oxygen saturation alone does not establish adequate ventilation.²⁹
- ☐ **Neuromuscular recovery:** If a blocker was used, confirm quantitative TOF ratio ≥ 0.9 at the adductor pollicis. Document the assessment and reversal drug/dose/time when given.²⁶
- ☐ **Physiologic readiness:** Hemodynamics and rhythm acceptable, temperature appropriate, major metabolic problems addressed, and pain managed without excessive respiratory depression.²⁹
- ☐ **Airway and team ready:** Position appropriately, clear blood/secretions, confirm any throat pack is removed, and assess swelling or other obstruction risks. Oxygen, suction, mask ventilation, rescue equipment, and skilled help must be available.²⁹

Remove the tube only when the preceptor agrees

- **Suction before extubation:** Gently clear the mouth/pharynx as indicated, ideally under direct vision. Coordinate stimulating suctioning with anesthetic depth; avoid repeated blind deep suctioning in a lightly anesthetized patient.²⁹
- **Deflate, remove, reassess:** When criteria are met and the team is ready, fully deflate the ETT cuff and remove the tube using the preceptor's technique. Provide mask oxygen and assisted ventilation as needed, watching chest movement, airway patency, respiratory effort, and oxygenation.
- **Stay with the airway:** Do not move on merely because the tube is out. Confirm sustained effective breathing and airway maintenance. If obstruction, laryngospasm, hypoventilation, or desaturation occurs, call for help immediately and follow the rescue plan.

Not a deep-extubation guide: Deep extubation is a separate, selected technique for experienced clinicians. An LMA also requires its own removal plan and device-specific instructions, not automatic use of the ETT sequence.²⁹

29. BJA Education, safe extubation: <https://pmc.ncbi.nlm.nih.gov/articles/PMC8606605/>

26. ASA, neuromuscular recovery: <https://csa-online.org/wp-content/uploads/2023/02/NMB23.pdf>



12

Transport: Keep the Patient Connected

The case is not finished when the tube comes out. Confirm that the patient is ready for transfer and that the destination can provide the needed care. A knowledgeable member of the anesthesia care team accompanies the patient and continues evaluation and treatment during transport.³⁰

Before leaving the OR

- **Reassess breathing:** Confirm an open airway and adequate ventilation after extubation. Continue oxygen and support appropriate to the patient. Unresolved airway or respiratory instability requires treatment and reassessment before routine transport.
- **Connect the oxygen device:** Place the planned nasal cannula or face mask and connect it to the transport oxygen source. Verify actual flow, a secure connection, and an oxygen supply adequate for the trip plus a reserve for delays. Check cylinder pressure and secure the tank in its approved holder.³¹
- **Keep rescue support available:** Have the required mask/bag ventilation equipment, airway supplies, and emergency medications. For a patient remaining intubated, verify the transport ventilator, settings, connections, battery, and oxygen supply.³¹
- **Move the IV safely:** Hang the bag on the stretcher/cart IV pole. Trace the line from bag to patient, protect the insertion site, and ensure tubing is free of the bed, wheels, rails, and transfer equipment.
- **Check the bag before moving:** Ensure enough prescribed fluid remains to maintain the planned infusion and support medication delivery during transport. Replace a nearly empty bag before departure when appropriate. You do not want to exchange a bag during an emergency. This is a readiness check, not a reason to give unnecessary fluid or disregard cardiac/renal restrictions.
- **Transfer deliberately:** Lock and align the stretcher and OR table, use the required assistance, protect lines and drains, and secure the patient with rails and safety measures once on the stretcher.
- **Continue monitoring:** Use transport monitoring and support appropriate to the patient, anesthetic effects, and facility policy; do not interrupt necessary monitoring. Observe breathing and consciousness as well as the monitor. Check that required monitors and pumps have sufficient battery power.^{30,31}

On arrival

Connect the receiving oxygen supply and monitors, verify flow and readings, and reassess the patient. Record arrival vital signs and oxygen/airway support. Stay until the receiving nurse has accepted nursing responsibility and any immediate concern has been communicated and addressed.³⁰

30. ASA, postanesthesia care: <https://www.asahq.org/standards-and-practice-parameters/standards-for-postanesthesia-care>

31. APSF, intrahospital transport: <https://www.apsf.org/article/intrahospital-patient-transport-checklists-adverse-events-and-other-considerations-for-the-anesthesia-professional/>



12

PACU Handoff and Closing the Case

Give a focused, complete report while the receiving RN can listen and ask questions. I emphasize pertinent history and what actually happened during the case, rather than reciting the entire chart. Use the facility's handoff format and make the recovery priorities clear.³⁰

Report what the PACU team needs next

- **Patient and procedure:** Identity, procedure/site, allergies, relevant baseline status and medical conditions, and the anesthesia technique. Include OSA/CPAP, diabetes/glucose, cardiopulmonary concerns, or other risks that affect recovery.
- **Relevant substance history:** Communicate smoking, alcohol, drug use, chronic opioid use, or treatment for opioid use disorder when relevant to respiratory risk, analgesia, tolerance, or withdrawal. Use factual, nonjudgmental language.
- **Airway and breathing:** Ease or difficulty of mask ventilation and intubation, device used, extubation course, current oxygen needs, and any airway concern. Report the blocker, reversal agent/dose/time, and documented quantitative recovery when applicable.
- **Course and medications:** Arrhythmias, hypotension, respiratory events, other complications and their treatment; opioid names, total doses and last-dose times; other analgesics/blocks, antiemetics and times, antibiotics, and relevant ongoing infusions.
- **Fluid balance and recovery plan:** Fluids/blood, estimated blood loss, urine output if measured, IV access/lines/drains, temperature, relevant labs, current vitals, and pending tasks. Verify pain/nausea orders, special precautions, and who to call for concerns.

Document, reconcile, and complete

- **Close the clinical loop:** Document the receiving RN, handoff and acceptance of care, arrival vitals, patient condition, oxygen support, and required case events. Complete the anesthesia record and required postanesthesia evaluation with the preceptor; an electronic “case closed” button is not a substitute for required care.³⁰
- **Account for every controlled drug:** Reconcile the amount withdrawn with the amount administered, returned, and wasted. Confirm drug, concentration, units, and actual remaining quantity against the medication record. Investigate discrepancies with the preceptor/pharmacy rather than altering entries merely to make totals match.³²
- **Waste promptly and correctly:** Waste unused controlled drugs immediately or as soon as safely practicable through the approved process, with the required authorized witness and documentation. Keep them secure until then. Never leave the patient unattended or interrupt an essential handoff to complete waste.³²

30. ASA, postanesthesia care: <https://www.asahq.org/standards-and-practice-parameters/standards-for-postanesthesia-care>

32. AHRQ PSNet, anesthesia medication waste safety:
<https://psnet.ahrq.gov/web-mm/multiple-high-risk-events-involving-workflow-wasting-medications-used-anesthesia>



13

Quick-Reference Pocket Card

Meet the team

Introduce yourself to the circulating RN and other OR staff. Give your name, spelling, and learner role for the intraoperative record.

SOAP-ME

Suction • Oxygen and backup ventilation • Airway • Pharmacy • Monitors • Equipment.

Personally verify readiness

Confirm the machine-specific checkout even if the anesthesia tech has completed the initial check. Verify suction, agent level/filler ports, circuit, monitors, alarms, and backup equipment.¹

Prepare the airway

Laryngoscope • Correct-size ETT • Tested cuff • Syringe and stylet • Clear tape • Oral airway and tongue depressor • Backup airway • Postoperative oxygen device.

Set out monitors and supplies

ECG leads/electrodes on the bed • Pulse oximeter • BP cuff and hose/cable • Appropriate temperature probe • Capnography • Gloves • IV equipment and additional lines.

Think about position and warming

Select the Bair Hugger blanket for the surgical site. For prone cases, have the planned head/face support and flexible breathing-circuit extender ready.

Prepare and secure medications

Prepare case-specific medications and label each syringe. Verify emergency stock. Keep controlled substances secure under policy; do not carry loose prepared syringes in pockets.²

Confirm before the case

Oxygen? Agent? Absorbent? Circuit/leak check? Monitoring? Alarms? Know where the difficult-airway cart, crash cart, MH cart, and emergency drugs are, and how to call for help.

1. APSF, checkout: <https://www.apsf.org/article/new-guidelines-available-for-pre-anesthesia-checkout/>

2. ASA, labeling:

<https://www.asahq.org/standards-and-practice-parameters/statement-on-labeling-of-pharmaceuticals-for-use-in-anesthesiology>



Key References & Disclaimer

How to use the citations: Numbered superscripts refer to the numbered source notes printed at the bottom of the same page. The unnumbered reading list below is separate; it does not restart or replace those footnote numbers. Full source URLs are clickable throughout the module.

Core clinical guidance

APSF: pre-anesthesia checkout principles

<https://www.apsf.org/article/new-guidelines-available-for-pre-anesthesia-checkout/>

ASA: basic anesthetic monitoring

<https://www.asahq.org/standards-and-practice-parameters/standards-for-basic-anesthetic-monitoring>

ASA: monitoring and antagonism of neuromuscular blockade

<https://csa-online.org/wp-content/uploads/2023/02/NMB23.pdf>

ASA: postanesthesia care

<https://www.asahq.org/standards-and-practice-parameters/standards-for-postanesthesia-care>

ADA: hospital and perioperative diabetes care, 2026

<https://pmc.ncbi.nlm.nih.gov/articles/PMC12690180/>

Original-module background reading

ProCRNA: the preanesthetic setup

<https://procrna.com/clinical-topic-the-preanesthetic-set-up/>

Anesthesia Key: anesthesia machine checkout

<https://aneskey.com/anesthesia-machine-checkout/>

NARs: tips for SRNAs starting clinicals

<https://www.youtube.com/watch?v=ellLq8KZBEA>

Background reading provides context, not a current workstation operating procedure. Use the exact manufacturer operator instructions, applicable drug labeling, current professional guidance, and local policy at the point of care.

Educational scope: This module is an orientation aid for supervised learning. It does not replace your anesthesia program's curriculum, institutional policy, manufacturer instructions, or the preceptor's direct supervision. Independently verify all medication doses and equipment settings before use. Editorial review is not independent clinical approval of every retained drug reference.

Edition: October 2026. Created for CE HALO LLC. Positioning diagrams are original educational schematics, not to scale.